

SECOND AMENDED STATEMENT OF FACTS AND CONTENTIONS (FORM 9A)

WC/2024/227 - Cory Lea Shepherd (Appellant) v Workers' Compensation Regulator (Respondent). Section 549, Workers' Compensation and Rehabilitation Act 2003. The numbers in square brackets, for example [¶ 40], are the paragraph numbers of the Appellant's notice to admit facts served on the Respondent on 28 August 2026. The Respondent responded to that notice on 8 September 2026.

PART A

Appellant: Cory Lea Shepherd. Employer: State of Queensland (Queensland Health) - Logan Hospital Switchboard. Role: Administration Officer, Switchboard Services, classification AO3, a continuous shift working role. [¶¶ 1 to 4, 13] Three movement forms approved in 2026 record the shift arrangement as "Continuous Shift Worker", and the Respondent does not allege that the Appellant ceased to be a continuous shift worker at any time before 18 June 2024. [¶¶ 14 to 16] Injury: major depressive disorder with anxious distress (DSM-5 296.23). The treating psychiatrist recorded the diagnosis in writing on 24 October 2024 and gave it the code DSM-5 296.23 in his report of 13 February 2025. The injury is a diagnosed medical condition. It arose from the Appellant's employment over the course of conduct pleaded at Part B.2 below, between about June 2023 and June 2024. It is not pleaded as arising from an event on a single day. The date of injury is 18 June 2024, as recorded on the work capacity certificates. The injury is pleaded as arising from that course of conduct, not from a single event on that day.

PART B - MATERIAL FACTS

Each fact set out below that carries a paragraph number in square brackets was admitted by the Respondent, to the extent recorded in its response of 8 September 2026. A statement without a bracket is drawn from a document identified in Part B.7 or is the Appellant's own evidence; the register at Tab 31, the call statistics and the account of travel and sleep in Stressor 3 are of that kind, and facts 228 to 231 were not admitted. The Commission is asked to decide from the admitted facts what the circumstances were, whether they were reasonable, and how much they contributed to the injury.

How this Part is arranged. Each stressor is pleaded so that it can be decided on its own facts. Where one admitted fact matters to more than one stressor, it is repeated on purpose. The repetition does not allege separate injuries and does not count causation twice. It shows the different part the same event plays in each stressor within the whole course of conduct. The medical material is placed between the facts only to show what was reported at the time, the diagnosis, capacity and treatment history. It is not used to turn a clinical history into proof of an event that is otherwise disputed.

1. The starting point - the medical record

1.1 The Switchboard receives emergency response notifications and distributes them to the response groups by category of emergency, and maintains the contact and paging information by which that is done. [¶¶ 5, 7 to 8, 10] The position is also required to work effectively with limited supervision, to meet deadlines and establish work priorities, and to work as a member of a multi-disciplinary team in a changing environment. [¶ 11]

1.2 **Prior health.** Before 18 June 2024 the Appellant had not been diagnosed with or treated for depression. The general-practice record of 16 November 2023 (Dr Nanayakkara) records poor sleep with shift work, that the Appellant could not do shifts without a good sleep, "No psychological illness such as depression/ psychosis", and mood good, with melatonin and temazepam prescribed that day. [Tab M1 - general-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024, obtained by the Respondent under the Form 29 signed 4 July 2025; Respondent's item 11.] This record is relied on because it shows, before any claim or proceeding, that shift work was affecting the Appellant's sleep, and what was prescribed for it. What it means clinically is a matter for the treating doctors. Private entries unrelated to the injury are redacted on the extracted pages and marked as such. The records show no diagnosis of, or treatment for, a depressive disorder before June 2024. No medication for mental health was current in May 2024 (paragraph 1.3). On the same records the Respondent's own review found that employment was a significant contributing factor to the injury (paragraph 1.10). Section 32(1) requires that employment be a significant contributing factor, not the only one.

1.3 The referral of 16 May 2024. The referral letter of 16 May 2024 (Dr Zhao) renews a referral to a psychiatrist, Dr Amini, for "ongoing care and management". It lists the past medical history as the history-list items "26/10/2022 ADHD" and "26/10/2022 Anxiety". It lists the medications then current; they include no antidepressant, no anti-anxiety medication and no other medication for mental health. [Tab M1.] No appointment with the psychiatrist was obtained on that referral. The referral was made the day after the request to retract at Stressor 1(i). [¶¶ 76 to 77]

Particulars: paragraphs 76 to 77 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.4 Deterioration and first presentation. The Appellant last worked a shift on 3 June 2024. The date of injury recorded on the certificates is 18 June 2024. On 28 June 2024 (Dr Slawinski) the record shows "stress at work" and "upset by people not following rules", reason for visit anxiety. On 1 July 2024 (Dr Hawes) the record shows "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "causing anxiety". [Tab M1.] The events those records name are the matters admitted on 8 September 2026 and pleaded at Part B.2 below. The records are relied on as the record, made at the time, of what the Appellant reported before any claim decision, dismissal or proceeding. They are not relied on as a finding of how the injury was caused.

1.5 Certification. The work capacity certificates of Dr Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024, state the date of injury as 18 June 2024, that the Appellant was first seen for this injury on 1 July 2024, and certify no functional capacity continuously from 1 July to 6 October 2024. The referral to a psychiatrist is recorded on the certificate of 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates". [Tab M2; Respondent's items 7 and 8.] The medication box is unticked on each certificate.

1.6 Diagnosis, 24 October 2024. At the first consultation, on the referral, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". The Appellant's email of 5:12 pm that day reads "I have finally been able to see a psychiatrist today". [Tab M3; Respondent's item 9.] Read with paragraph 1.5, it records how the injury certified as anxiety and stress on 1 July 2024 had become a depressive disorder by 24 October 2024.

1.7 The report of 13 February 2025. Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). The report states the severity, the effect on functioning and the treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician records the origin as "workplace stress stemming from issues with management and rostering", records delayed pay as a source of financial stress, and states that "premature exposure to the workplace is more likely result in significant deterioration". [Tab M4; Respondent's item 10.] The stressors recorded in that report match matters admitted on 8 September 2026: management and rostering, and the night-shift line; the shorter break; and pay withheld or delayed from the 5 February 2024 fortnight, uncorrected at 13 May, "claims older than 3 months" on 28 May, a claim effective 30 March recorded "Part Completed" on 30 May, and the two February claims absent from the myHR report.

Particulars: paragraphs 185 to 190, 193, 196 to 197, 203, 210 to 212, 220, 258 to 260 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.8 The report's footer, and the author's position. The report bears a footer reading "for the only reason of clinical information and not for medico-legal use". The author's emails of 5 and 8 September 2026 state of his records "You can use them according to the need to support your legal issues", and identify the report of 13 February 2025 as "the report that captures the relevant information you have requested" on the matters in issue. [Tab M5.] The report is not relied on to say which of the individual events pleaded at Part B.2 caused the injury, or how much each contributed. Nothing after 24 October 2024 is relied on as a cause of the injury.

1.9 Current capacity. The Employee Capability Checklist completed by Dr Day Hong Ma on 3 July 2026 records current capacity and restrictions and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors". [Tab M7.] It is relied on for the effect of the injury and for capacity only, not for cause. It is dated after 1 July 2024.

1.10 Records still to come, and the finding in the decision under appeal. The clinical records of the treating psychiatrist from 24 October 2024 were offered by the practice on 5 September 2026 and have been requested. They are not yet received and will be served on receipt. On the medical evidence then before it, the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a psychological nature" and stated: "Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury". The claim was rejected under section 32(5), not section 32(1). [Tabs M6, M8 and M9.] The hearing is de novo (a fresh hearing), so that finding does not bind the Commission. It is relied on as an admitted document.

Particulars: paragraphs 261 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.11 The medical evidence at hearing. No medico-legal report has been, or will be, prepared for this proceeding. The Appellant relies on the treating records and documents at Tabs M1 to M4 and M7 to M9, served with the Schedule of Medical Documents Relied Upon on 9 September 2026. He also relies on the oral evidence of the treating general practitioner, Dr Peter Hawes, and the treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah. Each is named in the Appellant's list of witnesses filed 9 September 2026. Each will be asked to address, from his own records and the admitted facts put to him: the diagnosis; the history reported to him and when; whether the Appellant's employment, and in particular the matters pleaded at Stressors 1 to 3, was a significant contributing factor to the injury; and the non-employment matters recorded in the report of 13 February 2025. This paragraph gives notice of that evidence. Once hearing dates are set, the Appellant will apply for attendance notices and for leave for each practitioner to give evidence by telephone or video link at a fixed time.

2. The stressors that caused the injury (the whole course of conduct, June 2023 - June 2024)

In summary. The injury: a psychological injury, diagnosed, with employment found by the Respondent's own review to be a significant contributing factor [¶¶ 261 to 262]. Stressor 1: a continuous 24/7 patient-safety function [¶¶ 2, 8, 289] carried on while database access was removed and not restored [¶¶ 40, 55]; process changes made effective the day they were notified, with no listed record of consultation [¶¶ 51, 181, 273]; calls misdirected and the directory not corrected [¶¶ 58, 67, 89 to 93, 109 to 110]; and the manager's office hours not stated for nine months [¶¶ 71 to 73]. Stressor 2: pay wrong in four fortnights [¶¶ 185 to 189]; Payroll's direction of 3 May 2024 not actioned at 13 May [¶ 193], still awaited on 21 May [¶ 195], lodged 28 May and recorded "Part Completed" [¶ 203]; and a paid leave entitlement declined twice on attachments the Respondent admits were present [¶¶ 116, 118, 134]. Stressor 3: a seven-hour break rostered in that role [¶ 284] against a minimum of ten hours, or eight by written agreement [¶ 285], the agreement applying only to staff-initiated swaps and no swap alleged [¶¶ 225, 234]; the fatigue enquiry answered after 23 days with a refusal [¶¶ 247, 249]; no fatigue risk management at the Switchboard before 30 June 2024 [¶¶ 264, 269 to 271]; and the Respondent's own review finding that the rostering was unreasonable management action [¶ 260].

STRESSOR 1 - THE CONDITIONS IN WHICH THE WORK WAS CARRIED ON

(a) The duty to keep the contact information accurate. A key responsibility of the position is to collate information and maintain the Omnivista database and SharePoint so that the information held within Switchboard Services is accurate and appropriate. [¶ 6]

A further key responsibility is to support hospital staff in the allocation, coordination and fault repair of all pager units, including maintaining the database and registers with accurate and current information. [¶ 7] Each of (b) to (h) below is pleaded against that duty.

(b) The removal of the means of performing that duty, 18 July 2023. On 18 July 2023 at 12:56 pm the Switchboard Telecommunications Coordinator wrote to the Appellant and the Switchboard team explaining that her role was a temporary project role focused on fixing the database. [¶ 39]

She stated: "While I am fixing up the database and all of its entries, I will be removing everyone's access to the database." [¶ 40]

She stated that she had removed the Contact & Number Changes book out of the room, preferring all changes to be sent to her email. [¶ 45]

The Respondent's own review decision records the same matters. [¶¶ 46 to 48]

The Respondent does not allege that access to the database was restored to the Appellant at any time before 18 June 2024. [¶ 55] From 18 July 2023 to 18 June 2024 is eleven months. Nothing is alleged to have happened in that time to restore access. The duty at (a) remained on the position throughout that period.

(c) The repair path, and its availability. The Coordinator's stated working hours were "every Tuesday and second Monday, 8:00 - 16:00". [¶ 41] A person wanting an entry updated was to "contact me directly, either via email or extension". [¶ 42] If an entry was needed on a day she was not present, and was urgent, it was to be sent to the Switchboard Manager. [¶ 43]

A request made "after hours (overnights, on the weekend or public holiday)" would "have to wait until either Chloe or myself are back". [¶ 44]

How long an incorrect entry stayed uncorrected depended on when those two people were available.

(d) How critical the function is, as the Respondent admits. On 18 February 2026 the Respondent responded to an earlier notice to admit facts in this proceeding. [¶ 283] It admitted that "Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety". [¶ 289] In admitting that paragraph it added "and says that there was a procedure in place for this to occur", and it denied a further paragraph on the ground that there was already a procedure in place. [¶¶ 290 to 291] The procedure in place from 18 July 2023 is the procedure pleaded at (c) above. Its operation between 2 and 20 May 2024 is pleaded at (g) and (h) below.

(e) The further changes to process, April and May 2024. On 15 April 2024 at 12:39 pm the Switchboard Manager wrote to Logan Switch, copied to the Director and the Switchboard staff, with the subject "Afterhours Oncall Process - Switchboard". [¶ 49] That email stated that the process during office hours remained the same and that staff were to contact her through switch, the office or her mobile, and: "You will see that Ellen and myself have added Afterhours on call, the days that are highlighted in purple show who is on call after hours." [¶¶ 50, 52] It stated: "This new process is effective from today." [¶ 51]

She signed it as "A/Switchboard Manager". [¶ 53]

The review decision records that on 19 April 2024 she emailed the team stating that due to errors being made with respect to data entry a new process was to be followed which included more checks to ensure accuracy of data entry. [¶ 54]

On 13 May 2024 at 4:29 pm the on-call roster for the fortnight beginning that day was circulated to the Switchboard team. [¶¶ 111 to 112]

From 14 May 2024 the process for notifying unavailability for a rostered shift was also directed through the same person, both emails identifying the Switchboard as a means of contacting her. [¶¶ 162 to 165]

The Respondent does not allege that the Appellant was informed before 14 May 2024 that any other means of notification was required. [¶ 166]

(f) The duty to consult, and the absence of any record of consultation. Clauses 3.2.3 to 3.2.5 of the Queensland Public Health Sector Certified Agreement (No. 11) 2022 provide that the requirement of consultation is never to be treated perfunctorily or as a mere formality. They provide that consultation involves more than a mere exchange of information, and requires participants to contribute to the decision-making process "not only in appearance, but in fact". They provide that the process requires the exchange of timely information so that the parties have "an actual and genuine opportunity to influence the outcome, before a final decision is made". Clause 4.1.4 requires a business case to be tabled for consultation for significant organisational change, including major alterations to current service delivery arrangements; it is relied on only to the extent the change was of that kind. The change notified on 15 April 2024 was stated to be effective the day it was notified. [¶ 51]

The Respondent's amended List of Documents records no document recording consultation with Switchboard operators before that change. [¶ 273]

The Respondent does not allege that agreement under clause 6.2 of the Award was obtained, or that a ballot of affected employees was conducted, before it. [¶¶ 180 to 181]

The employer conducted that process on another occasion. Its Consultation outcome document defines agreement as the consent of a majority of affected employees, and records that seventeen employees were invited to vote, seventeen votes were received, and the proposal was accepted. [¶¶ 167 to 170, 176 to 177]

On 20 May 2024 at 4:30 pm the Switchboard Manager wrote: "Taking note of your recommendation, we can also put the updated procedures out to the team for consultation before implementing." [¶ 101]

(g) What arrived at the console: the misdirected emergency calls, 2 to 9 May 2024. A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; to maintain call queues to a minimum at all times; and to operate under pressure, particularly where high volume call traffic is concerned. [¶¶ 5, 8, 10]

The console took about 270 to 440 calls on an eight-hour shift (Stressor 3(h)(v) below). Emergency codes were received and paged through it throughout the day (Stressor 3(h)).

On 3 May 2024 at 3:06 pm the MASPER Registrar wrote to the Switchboard Manager with the subject "Switchboard issues 2nd and 3rd May 2024", listing five occasions on 2 and 3 May. [¶¶ 56 to 57] Of the occasion at 14:46 on 3 May the email records: "MET call team called x5290 asking where MET call was located 'VHUB' - switchboard could not tell them where VHUB was. Had to be redirected by MASPER". [¶ 58]

On 8 May at 5:28 pm she wrote again, listing four further occasions, recording of those on 8 May that a call was "incorrectly put through to MASPER" on each occasion. [¶¶ 59 to 61] Those nine occasions fell while the arrangement notified on 15 April 2024 was in force. [¶ 68]

The first reply was sent on 9 May at 9:20 am, and asked Dr Wong, to whom the reports had been copied, to confirm her business hours because they were "not provided on the rosters", noting that her dect phone was "currently switched off". [¶ 65]

That reply was sent five days, eighteen hours and fourteen minutes after the first email. [¶¶ 62 to 64]

At 10:15 am the same day the Switchboard Manager wrote to Logan Switch introducing a new process and referring to "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". [¶¶ 66 to 67]

The Respondent does not allege that any communication was sent to Logan Switch or to the Switchboard staff concerning those occasions at any time after 3 May 2024 and before 10:15 am on 9 May 2024. [¶ 69]

So the sequence at the console was: the first report on 3 May 2024, a second on 8 May (the fifth calendar day after the first), nine occasions in all, no communication to the Switchboard staff about them for five days, eighteen hours and fourteen minutes, and then a new process on 9 May (the sixth day). [¶¶ 56 to 69]

(h) What arrived at the console: the Integrated Respiratory Service, 15 to 20 May 2024. On 15 May 2024 at 11:47 am an Administration Officer of the Integrated Respiratory Service wrote to Logan Switch, marked of High importance: "Could you please amend your number registry/directory to show #8768 belongs to the Integrated Respiratory Service... We are not Respiratory Medical OPD and we do not have any doctors working out of this area." [¶ 89]

The Respondent does not allege that any response was made to that email before 20 May 2024. [¶ 90]

On 20 May at 11:03 am she wrote again, marked of High importance: "Just a courtesy reminder, we continue to get calls put through to us for Respiratory Medical Outpatients... we can not help patients or other clinical staff with OPD issues." [¶¶ 91 to 92]

The Respondent does not allege that any response was made to that email before 2:05 pm that day. [¶ 93]

At 2:05 pm, from the Logan Switch account and within the office hours stated to all Switchboard staff on 17 May 2024, the Appellant wrote identifying the cause as modifications made to the document "Outpatients Department - Clinic contact Details" on 22 February 2024, and recommending a modification and review of that document. [¶¶ 94 to 96] That email was sent three hours and two minutes after the Service's second email, and five days, two hours and eighteen minutes after its first. [¶¶ 97 to 98]

The Respondent does not allege that the Appellant was rostered to work after 2:05 pm that day. [¶ 99] The reply at 4:30 pm stated: "Thank you for bringing this to my attention however this task was being actioned. I had discussed with Richard this morning about the update of outpatients respiratory/medical." [¶¶ 100, 104] It was sent two hours and twenty-five minutes after the Appellant's email, and two hours after the conclusion of the office hours stated to all Switchboard staff on 17 May 2024. [¶¶ 102 to 103]

The Respondent's amended List of Documents records no document created on 20 May 2024 recording that discussion, and no communication sent by either named person to the Service on that day; and the Respondent does not allege that the other named person communicated with the Service on that day. [¶¶ 105 to 108]

The Respondent does not allege that the contact-details document was amended on or before 20 May 2024, or that the Switchboard staff were notified of the modifications made to it on 22 February 2024. [¶¶ 109 to 110]

So the sequence was: the Service's first email on 15 May 2024; its second on 20 May, the fifth calendar day; the Appellant's escalation five days, two hours and eighteen minutes after the first email and three hours and two minutes after the second; and the reply two hours and twenty-five minutes after that, two hours after the stated office hours had ended. In that time the Respondent alleges no response to the Service, no amendment of the document, and no notice to the staff of the change made to it on 22 February 2024. [¶¶ 90, 93, 97 to 98, 102 to 103, 105 to 110]

(i) The office hours of the manager to whom the position reports. The position reports to the Switchboard Manager, and the role description names that person as the contact for the position. [¶¶ 3 to 4]

On 23 August 2023 at 2:18 pm she wrote to Logan Switch with the subject "What's Chloe's Hours?!": "I am fulltime and work flex hours - You may have noticed recently I am starting at different times in the morning and finish at different times in the afternoon", and "My hours would range from starting between 6-9am and finishing 2-5pm." [¶¶ 70 to 72]

The Respondent does not allege that fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024. [¶ 73] That is nine months.

On 15 May 2024 at 1:15 pm the Appellant wrote to the Manager and Logan Switch, copied to the Switchboard staff, the Director and LBH_HR, asking that she share her office hours so the department could be aware of her regular schedule, noting inconsistency in arrival and departure times, and asking that "any directives and changes within the department is made in consultation with the team". [¶¶ 74 to 75]

At 6:23 pm the Director replied, copied to the Manager and marked of High importance, stating that his email "did not demonstrate our iCARE2 value of Respect and did not comply with our Code of conduct", and asked him to retract it. [¶¶ 76 to 77]

At 7:09 pm the Appellant replied that "Requesting clarity on business hours is a reasonable question". [¶ 78]

On 17 May 2024 at 9:30 am the Manager wrote to the whole department, headed "Switchboard Manager - On call and Hours", stating that her hours "can vary", that "I will be in later between 0800-830am. Otherwise my hours are from 06:30-14:30", that "I will not be in the office until around 11am on Monday 20.05.2024", and that "Moving forward so communication is clear for the team, I will be sending an email to switch to advise of any change to my office hours for the week". [¶¶ 81 to 84, 113] Those stated office hours concluded at 2:30 pm. [¶ 88]

On 21 May 2024 at 2:53 pm the Director replied that she would follow up on the issues raised, and asked "can I ask was there one or more particular changes and/or directives where you had concerns about consultation and communication?" [¶¶ 79 to 80]

On 18 June 2024 at 8:58 am the Manager wrote to Logan Switch: "I am taking today off, and hopefully will be back tomorrow", and "I am sorry I haven't been there for you all over the past week." [¶¶ 85 to 87]

So the request for the Manager's office hours was made at 1:15 pm on 15 May 2024. The request to retract it came five hours and eight minutes later, at 6:23 pm. The hours were stated to the whole department one day, twenty hours and fifteen minutes after the request, at 9:30 am on 17 May. The Director's question about which changes had concerned him came six days, one hour and thirty-eight minutes after the request, at 2:53 pm on 21 May. Before the request, no fixed hours had been stated for nine months. [¶¶ 73 to 84]

(j) The Communication Book, 6 June 2023. A key responsibility of the position is to maintain discretion and exercise judgement where necessary to resolve problems within the scope of the role, in situations where precedence has not been set and procedures are not defined. [¶ 9]

At 9:57 am on 6 June 2023 the Switchboard Manager wrote to Logan Switch that the book "is not used to simply put your point across or make any indirect comments towards the team". [¶¶ 148 to 149]

At 4:05 pm she wrote to the Director: "I was not aware of who put this entry in at the time but I took it out last week", and that she felt it was being used as a "burn book" more than a professional tool. [¶¶ 145 to 147]

The Respondent's pleading admits that pages were removed on or around 6 June 2023, and says the event was reported to the Director by email that day. [¶¶ 143 to 144]

On 18 February 2026 it admitted that a page was removed from the workplace communications book, and did not admit that the page contained the Appellant's handwriting "because the respondent does not have a copy of the page". [¶ 288]

The Respondent does not identify the entry removed or its author, does not allege that the removed page has been located or that any copy exists, does not allege that any entry made by the Appellant remains in the book, and does not allege that the book ceased to be in its possession during his employment. [¶¶ 150 to 153]

The Respondent has admitted that it does not have a copy of the removed page. [¶ 288] The Respondent's amended List of Documents of 14 August 2026 does not list the Communication Book, nor any page or entry from it; that proposition (paragraph 154 of the notice) was not admitted.

(k) The matters raised in August and September 2023, and the response. On 7 August 2023 at 12:21 pm the Appellant wrote to the Manager, the Director, Ms Conaghan and Ms Smith, marked of High importance, asking to "formally adhoc an additional 2 shifts per fortnight" under clause 11.7 of the agreement, and: "Please give me some space and stop with any further communication as I have had enough of it." [¶¶ 26 to 27, 155]

At 1:43 pm the Manager replied, apologising for the rostered shift and confirming it was "an oversight", and asking "Would you like me to roster you off tomorrow Tuesday 8th 0700-1500 to give you the required rest period". [¶¶ 156 to 157]

At 3:13 pm the Director replied. [¶ 158]

On 8 August 2023 at 4:15 pm the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

On 29 August 2023 she sent HR Policy E12, and on 4 September 2023 the Appellant replied. [¶¶ 160 to 161]

On 31 August 2023 he provided a written application headed "Request to Increase Working Hours to Full Time Rotational Roster", expressing willingness to take on additional night shifts. [¶¶ 30 to 31]

On 4 September 2023 he confirmed that he was able and willing to work any roster presented to him including the current schedule with full 24-hour availability, that he had no issues with shift work, and attached a draft roster spreadsheet. [¶¶ 33 to 35]

On 27 September 2023 the Manager wrote with the subject "Approved - Permanent Full Time FTE", stating she was happy for him to commence full-time hours from 16 October 2023. [¶¶ 37 to 38]

On 8 August 2023 he had confirmed that he would work the replacement shift and was looking to pick up another. [¶ 29] On 8 September 2023 the Director wrote that she was "glad to hear things seem to be going well with Chloe" and glad that he had applied for the additional shifts through the recent EOI. [¶ 36]

(l) The special pandemic leave application, February 2024 - the sequence. On 20 February 2024 the Appellant messaged the Manager that he had tested positive for COVID-19. She "requested that you send it to her so she could submit it in MyHR". [¶ 136] She had said he was eligible for up to five days' paid leave on photo evidence of the test. He sent it to her as she asked. She then asked him to submit the Special Pandemic Leave request through myHR himself and attach a statutory declaration or photo evidence of his test. [¶ 132] The myHR submissions report records the initiator of that request as the Appellant [¶¶ 140 to 141], and the Respondent does not allege that any other Switchboard employee was required to submit such a request personally through myHR in February 2024. [¶ 142] The leave request history for Process Reference 15480560 records him creating a draft at 11:24:48 that morning and submitting the request at 11:41:27. [¶¶ 114 to 115] So the material was provided to the Manager once, as requested, and then had to be submitted by him through the system: once that morning, and, after two

declines, twice more.

It records the request being declined at 12:23:43 on 21 February 2024; submitted again at 06:07:32 on 28 February; declined again at 09:06:07 on 29 February; submitted a third time at 11:07:28 that morning; and approved at 11:21:03. [¶¶ 116 to 120]

It records the Appellant as having submitted the request on three occasions and the Manager as having declined it on two, and records the Director as approving it at 16:05:08 on 1 March 2024. [¶¶ 121 to 123] The approval followed the third submission by thirteen minutes and thirty-five seconds; the interval from draft to final approval was ten days, four hours, forty minutes and twenty seconds. [¶¶ 124 to 125] Two reasons for the declines appear on the record and they are not the same. The review decision records that on 21 February the Manager messaged that she had to decline because the minimum period for COVID leave was five days for a positive staff member, that she noted the statutory declaration provided was sufficient evidence, and that she asked him to resubmit his leave days. [¶¶ 137 to 138]

It records the Appellant messaging that he was confused about why his amended request was declined, and that he had done everything as required. [¶ 139]

The Respondent pleads that she declined the application because on her assessment the required statutory declaration was not attached; that "a review indicates that in fact, the attachments were present on the appellant's submission"; and that this was human error on the background of high work demands. [¶¶ 133 to 135]

So the request was made on 20 February 2024 and finally approved on 1 March 2024: ten days, four hours, forty minutes and twenty seconds from draft to final approval, with two declines and three submissions in between. The third submission was approved in thirteen minutes and thirty-five seconds. [¶¶ 114 to 125]

(m) What the employer states about its own systems and records. A key responsibility of the position is to follow defined service quality standards and occupational health and safety policies and procedures relating to the work being undertaken, to ensure high quality, safe services and workplaces. [¶ 12]

By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] It stated that "All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant." [¶ 267] It stated that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268]

The Respondent does not allege that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

(n) The complaint of 13 May 2024, and the days that followed - chronology only. On 13 May 2024 the Appellant lodged a corrupt conduct complaint regarding clinical risks. The Ethical Standards Unit formally determined this constituted a Public Interest Disclosure (Admitted Fact: Form 24, para 20, admitted 18 February 2026). The content is not set out. The Respondent's disclosure of 11 June 2025 (Part B.7.5) contains, in this order: the complaint form, dated 13 May 2024 (pp 9 to 10); the Appellant's email of 15 May 2024 at 3:35 pm to the Ethical Standards Unit, the complaints mailbox and LBH_HR (p 8); a forward of that email by LBH_HR at 3:41 pm the same day to three Human Resources officers (p 7); and an email of 16 May 2024 at 11:43 am from one of those officers to the Director and Ms Smith, copied to three others, attaching the complaint form (p 7). The same bundle contains the Appellant's email of 15 May 2024 at 1:15 pm (p 6) and the Director's reply at 6:23 pm asking him to retract it (p 5), pleaded at (i) above. [¶¶ 74 to 77] The Respondent's outline of Ms Taylor's evidence states that, acting on HR's advice, she removed the email of 15 May 2024 from the shared Switchboard inbox (Part B.6.1). On 21 May 2024 at 2:53 pm the Director asked which changes or directives had concerned him. [¶¶ 79 to 80]

(o) Matters pleaded as context only. The Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions does not identify the management action relied upon. [¶ 303] The matters at (j), (k), (l) and (n) above, and at paragraph 3 below, are pleaded as part of the sequence of matters the Appellant raised and the responses he received. They are not relied on as separate causes within Stressor 1. Where an admitted fact from that sequence is repeated under Stressor 2 or Stressor 3, it is relied on there only for the purpose stated under that stressor. The circumstances relied on as causes in Stressor 1 are those at (a) to (i) and (m).

The medical link - Stressor 1. Tab M1 records the 28 June 2024 presentation as "stress at work" and "upset by people not following rules". Tab M4 records the treating psychiatrist's later account of origin as workplace stress involving management and rostering. Those records are relied on as evidence, made at the time and by the treating doctors, of what was reported, the diagnosis and the effect. They are not relied on to prove that the events in Stressor 1 happened. Those events are proved, so far as they are relied on here, by the admitted facts and the employer documents identified.

STRESSOR 2 - REMUNERATION

(a) The basis of the entitlement. The position is a continuous shift working role over the full 24-hour period, seven days a week, and under mandatory requirements the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶¶ 2, 13]

(b) Public holidays on which the Appellant was not required to work. The review decision records that section 23 of the Award provides that where an employee in receipt of the additional week's leave prescribed by clause 19.2(a) is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, that employee is to be paid an additional day's wage or granted a day's holiday in lieu. [¶ 241] As at 1 May 2024 payroll was "still reviewing your entitlements regarding public holidays not required arising while you were a part-time employee". [¶ 255] The Manager confirmed "that since your commencement of full-time employment, all payments for public holidays not required had been processed". [¶ 256]

(c) The correction sought, and where each direction sent it. The Respondent's disclosure contains an email thread with the subject "Corey Shepherd 388372 Pay issues", the first message being from PayrollMetroSouth on 3 May 2024 to the Manager, copying the Appellant, bearing the signature block of a Client Service Officer of the Metro South Payroll Team. [¶¶ 182 to 184] That message identifies four fortnights requiring correction and states: "Please submit an AVAC to correct these shifts for each fortnight so Cory is paid corrected and his RDO balance will then be amended." [¶¶ 185 to 190]

On 10 May 2024 at 2:21 pm the Appellant wrote to PayrollMetroSouth and Ms Grant. [¶ 191]

On 13 May 2024 at 8:21 am Payroll replied to the Appellant: "I cannot see that any of the issues below have been corrected. Please speak to your Line Manager to have them corrected with an AVAC submitted through My HR". [¶¶ 192 to 193]

On 21 May 2024 at 12:33 pm the Manager wrote that she was "still I am waiting payroll confirmation". [¶¶ 194 to 195]

On 28 May 2024 at 8:36 am she wrote: "Please find attached Validation of claims older than 3 months, please sign and return to be as soon as possible so I can escalate for delegate approval." [¶¶ 196 to 197]

(d) Who could submit the correction. The myHR submissions report records seven submissions for the Appellant in the period 1 February to 31 May 2024, of which five were Attendance Variation and Allowance Claims. [¶¶ 198 to 199]

It records the initiator of each of those five as the Manager, and the initiator of no such claim in that period as the Appellant. [¶¶ 200 to 201]

(e) The state of the record, and the Respondent's pleaded position. The report records a claim with process number 16328886 submitted on 15 May 2024 and completed the next day. [¶ 202]

It records a claim with process number 16450619 submitted on 28 May 2024, with an effective date of 30 March 2024 and a processing date of 30 May 2024, with a status of "Part Completed", and records the status of every other such claim in the period as "Completed". [¶¶ 203 to 204]

It records the process numbers and submission dates in the period, and in it process numbers increase as submission dates increase. [¶¶ 205 to 206]

The Respondent pleads that "any discrepancies or errors were remedied in a timely manner" and that "there are no outstanding underpayments for the appellant", and that the Manager submitted the AVAC on 28 May 2024. [¶¶ 207 to 209]

Neither AVAC PRN 15397775 nor AVAC PRN 15605601 appears in the myHR submissions report. [¶ 210]

On 18 February 2026 the Respondent admitted the Payroll instruction of 3 May 2024, the fact that the Manager submitted the AVAC on 28 May 2024, and the email of 21 May 2024 stating that she was waiting for payroll confirmation. [¶¶ 296 to 298]

(e)(i) Each incorrect shift, when it was raised, and when it was corrected. The shift of 9 February 2024 was recorded as seven hours instead of eight, so the fortnight commencing 5 February 2024 was under the contracted hours and was topped up from the Appellant's RDO balance. [¶¶ 185 to 186] The shift of 28 February 2024 was recorded the same way, with the same result for the fortnight commencing 19 February 2024. [¶ 187] The fortnight commencing 18 March 2024, which contained the shift of 18 March, was recorded with too many ordinary shifts, reducing wages, because one of the shifts needed to be overtime. [¶ 188] The fortnight commencing 1 April 2024 was over the contracted 76 hours, again requiring overtime. [¶ 189] The public holiday of 30 March 2024 was, on Payroll's later account, "submitted late".

The Appellant raised the pay for the period 8 to 31 March 2024 with the Manager on 8 April 2024. [¶ 242] On 9 April she replied that she had escalated the fatigue enquiry to Human Resources. [¶ 244] On 24 April he wrote that it had been more than two weeks without any response or overtime payment. [¶ 246] On 1 May she replied that the fatigue payment for 18 March would not be processed, and that payroll was still reviewing his public holiday entitlements. [¶¶ 247, 255] On 3 May Payroll identified all four fortnights and directed the Manager to submit an AVAC for each. [¶¶ 185 to 190] On 10 May the Appellant wrote to Payroll. [¶ 191] On 13 May Payroll replied that none of the issues had been corrected and that the Line Manager had to submit the AVAC. [¶¶ 192 to 193] On 21 May the Manager wrote that she was still waiting for payroll confirmation and: "I will let you know PRN once it has been submitted." [¶ 195] On 28 May she sent the Validation of Claims form for signature and submitted the AVAC; it was processed on 30 May as "Part Completed". [¶¶ 196 to 197, 203, 209] He was not told that it had been submitted, or its process reference. His last day of attendance for a rostered shift was 3 June 2024; he was on leave from 7 to 12 June 2024; the date of injury recorded on the work capacity certificates is 18 June 2024 (Part B.1.4 to 1.5).

When each correction was made is stated by Payroll itself, in its email to the Respondent of 11 July 2025 and the payslips attached to it, disclosed by the Respondent in July 2025 (Disclosure from witness conferencing, Queensland Health Payroll, pp 2 to 3 and 19 to 24). The shifts of 28 February, 30 March and 1 April 2024 were adjusted on Payslip 06, for the pay period 13 to 26 May 2024, pay date 5 June 2024. That pay date was two days after the Appellant's last shift, after he had stopped attending work, thirteen days before the certified date of injury, and 33 days after Payroll's direction. On Payroll's account the 28 February adjustment was made by reversing the hours top-up and replacing it with an adjustment "which is leave without pay for the hour on the 28/02/2024 shift"; the 30 March entry was "amended to 'PubHolLv-DayPayforOffDay' (public holiday allowance)"; and the 1 April shift was amended to overtime.

The shift of 9 February 2024 was not corrected on that payslip. On Payroll's account it "was amended with an exemption for the Validation of Claims form on the 30.06.2025 and is on Payslip 07", for the pay period 9 to 22 June 2025, pay date 2 July 2025. That was 425 days after Payroll's direction of 3 May 2024 (the amendment itself being dated 30 June 2025); after the date of injury; after the Respondent's review decision of 24 October 2024; and after this appeal was commenced. It was made with an exemption from the form the Manager had asked the Appellant to sign on 28 May 2024. [¶¶ 196 to 197] The Respondent's pleading describes the pay issues as "remedied in a timely manner" [¶ 207], and its outlines of evidence describe them as "corrected" (Part B.6.3), without stating these dates.

So the pay sequence was this. The Appellant raised the pay for 8 to 31 March on 8 April 2024; the reply came the next day; his follow-up came fifteen days after that, on 24 April; and the refusal came seven days later, on 1 May. Two days after the refusal, on 3 May, Payroll identified all four fortnights and directed the Manager to submit an AVAC for each. From that direction: the Appellant wrote to Payroll after seven days (10 May); Payroll wrote that nothing had been corrected after ten days (13 May); the Manager wrote that she was still waiting after eighteen days (21 May); she submitted the AVAC after 25 days (28 May); it was

processed "Part Completed" two days after that (30 May); and three of the four corrections reached a payslip after 33 days (pay date 5 June 2024), two days after his last shift. From the shifts themselves to the payslip that adjusted them: 98 days for 28 February, 67 days for 30 March, 65 days for 1 April. The 9 February shift was corrected 423 days after Payroll's direction (30 June 2025) and reached a payslip after 425 days (2 July 2025): 509 days after the shift was worked. In all that time the Respondent alleges no step other than those listed. [¶¶ 185 to 197, 203, 209, 242 to 247]

(f) Paid Special Pandemic Leave - the same admitted sequence, relied on as an employment entitlement. The power to approve or not approve paid Special Pandemic Leave for the COVID-19 event was sub-delegated by the Acting Health Service Chief Executive to a Band 9 delegate, and the Instrument states that the power could not be further sub-delegated. [¶¶ 127 to 128] On 20 February 2024 the Manager asked the Appellant to send his material to her so she could submit it in myHR [¶ 136]; he did so; she then asked him to submit it through myHR himself [¶ 132], and the leave history records him creating the draft and submitting the request that morning. [¶¶ 114 to 115] The history records the request being declined on 21 February, resubmitted on 28 February, declined again on 29 February, submitted a third time at 11:07:28 that day and approved at 11:21:03. [¶¶ 116 to 120] It records the Appellant as making three submissions, the Manager as declining twice, the Manager as "Reviewer" and the Director as "Manager", with the Director recording final approval on 1 March 2024. [¶¶ 121 to 123, 126] The review decision records that on the first decline the stated reason was the minimum period for COVID leave while the statutory declaration was said to be sufficient; it records the Appellant saying he was confused by the second decline and had done everything required. [¶¶ 137 to 139] The Respondent's pleading states that the later decline was because the statutory declaration was assessed as not attached, but that review showed the attachments were in fact present, and pleads human error against a background of high work demands. [¶¶ 133 to 135] The Respondent does not allege that any other Switchboard employee was required personally to submit such a request through myHR in February 2024. [¶ 142] This sequence is repeated on purpose from Stressor 1(i). In Stressor 2 it is relied on as the handling of a paid employment entitlement: an entitlement that took three submissions and a decision process the Appellant could not complete by himself. It is not relied on as a separate injury.

(g) Rostering practices raised with Human Resources. On 20 May 2024 at 4:07 pm the Director wrote to LBH_HR: "I am just following up on this query I raised a little while ago in relation to a email I had received from a staff member about our rostering practices. Thus if someone might be able to give me a call regarding these staff concerns, that would be great." [¶ 222] That email attached the document "qh-gdl-401-3.3", the Queensland Health Fatigue Risk Management Systems Implementation Guideline. [¶ 223]

(h) The Respondent's pleaded answer to this stressor. The Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions, which contends that any management action was reasonable management action taken in a reasonable way, does not identify, by particular, date, document or cross-reference, the management action relied upon. [¶¶ 238, 303]

The medical link - Stressor 2. Tab M1 records the 1 July 2024 report of "work stress" and that pay was withheld at times and overtime was not processed. Tab M4 records the treating psychiatrist's later history of delayed pay as a source of financial stress. The medical records are relied on for what was reported and for the clinical course. The pay events themselves are proved by the admitted payroll, myHR and pleading facts set out above.

STRESSOR 3 - THE ROSTER AND FATIGUE

(a) The rostering history, and the Appellant's proposals. On 7 August 2023 the Manager acknowledged "an oversight" in the roster and offered to roster the Appellant off the following day "to give you the required rest period". [¶¶ 156 to 157]

On 18 February 2026 the Respondent admitted that the Director stated in an email dated 7 August 2023 that there had been a rostering error that was accidentally made. [¶ 287] The Appellant applied in writing to increase his hours to a full-time rotational roster, confirmed he was able and willing to work any roster presented to him, expressed willingness to take additional night shifts, and provided a draft roster spreadsheet. [¶¶ 30 to 31, 33, 35]

On 8 August 2023 the Director wrote regarding an additional shift to replace the shift not worked due to a rostering error. [¶ 28]

(b) The rostering errors acknowledged, and the decision withheld. On 26 April 2024 at 1:52 pm the Director wrote with the subject "Roster Concerns", referring to a meeting on 16 April 2024 at which the Appellant had raised a roster line ending with night shifts followed by three days off and then returning to night shifts, and stating that the Manager "was working to fix this error and would get in touch with you about what alternative shifts she could offer". [¶¶ 211 to 212]

The Respondent does not allege that she got in touch with him about alternative shifts following that meeting. [¶ 213]

On 1 May 2024 at 1:18 pm the Appellant referred to section 4, clause 10.4.1 of the Operations Manual, which "includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act", and stated: "I forwarded the toolkit to Chloe last week for review and action but have yet to receive feedback". [¶¶ 214 to 215]

The Respondent does not allege that the toolkit was reviewed or that any feedback was provided before 18 June 2024. [¶ 216]

On 8 May 2024 at 9:08 am the Director replied that she was following up with HR for further advice and would get back to him as soon as possible. [¶ 217]

On 10 May 2024 at 2:08 pm she wrote to Human Resources forwarding his email, stating: "I think he is trying to raise that he has concerns over how his manager is rostering for the Switchboard team and how it is impacting on staff fatigue, or more specifically his fatigue." [¶ 218]

She stated that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219]

She stated: "I acknowledge there has been a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶ 220]

She stated: "Cory's roster will not be considered, however Cory is not yet aware of this as we cannot advise what is the roster we are planning on going with before we send out the implementation plan for this business case for change". [¶ 221] Eleven days later, on 21 May 2024, she asked him which particular changes or directives had concerned him. [¶ 80]

(c) The Respondent's pleaded position on the roster. On 18 February 2026 the Respondent denied paragraphs of the Appellant's earlier notice on the stated grounds "because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable". [¶ 299]

The Consultation Paper of November 2024 states that the proposed roster "is intended to introduce a more equitable roster for Switchboard Services, Logan Hospital staff". [¶ 171]

It states that the proposed roster includes "Redistribution of nights for greater equity based on FTE" and a "Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties". [¶¶ 172 to 173]

It states that it includes an additional eight-hour Saturday shift to assist with call volume and the extension of a five-hour weekday shift to eight hours. [¶¶ 174 to 175]

(d) The shifts of 17 and 18 March 2024. The Appellant was rostered to finish at 23:00 on 17 March 2024 and to commence at 06:00 on 18 March 2024, a break of seven hours. [¶ 284] The shift commencing on 18 March fell on a Monday, the first shift of the working week. [¶ 227]

The Respondent does not allege that those consecutive shifts arose from a staff-initiated shift swap. [¶ 234]

The review decision records the Appellant's submission that the 8-hour agreement "was no longer fit for purpose" since his transition, and the surrounding matters as to that agreement. [¶¶ 17 to 25]

(e) The rest recorded between those shifts. The seven-hour figure is the interval between the rostered finish at 23:00 on 17 March 2024 and the rostered commencement at 06:00 on 18 March 2024. The review decision records the Appellant's response of 9 August 2024 that the break "did not include travel time" and that, with travel factored in, the break between shifts would have been less than five hours. [¶ 251] The Appellant's account is this. His home is on the Gold Coast and the drive to and from Logan Hospital took about an hour each way at those hours. Across the two shifts about four hours were spent driving. When

the 17 March shift ended at 23:00 he had been awake and working for about sixteen hours. Of the seven hours before 06:00, two were on the road and at best four to five were available for sleep. The seven hours and the response of 9 August 2024 are the admitted record; the rest of that account is the Appellant's own evidence.

(f) The standard, as admitted. The review decision records that the employer's own response included an extract of the Award stating that employees must be provided with a break of not less than ten hours between the termination of one shift and the commencement of another, and that eight hours applied instead of ten only in specific circumstances. [¶ 257]

The Award itself, clause 15.1(d), provides: "Employees must be allowed a break of not less than 10 hours between the termination of one shift and the commencement of another shift, except that 8 hours applies instead of the 10 hours in any of the following circumstances: (i) for the purposes of changing shift rosters by shift workers who rotate from one shift to another; or (ii) in any other case agreed upon between the employer and the employee concerned." Clause 18.10, on which the Respondent relies at paragraph 24(b) of its statement [¶ 237], is headed "Fatigue leave/rest period after overtime" and applies where overtime is worked. [Hospital and Health Service General Employees (Queensland Health) Award - State 2015]

On 18 February 2026 the Respondent admitted that "The Employer's 'Fatigue Risk Management Policy' and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement". [¶ 285] It denied a further paragraph of that notice on the ground that in June 2020 the Appellant signed an agreement allowing an 8-hour break between shifts. [¶ 286]

(g) The agreement of 17 June 2020, and its scope. By email of 7 July 2026 a Senior Consultant, Human Resources wrote that the rostering "currently provides more than" the required break, and acknowledged the 8-hour agreement signed on 17 June 2020 "which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred". [¶¶ 224 to 225]

The Respondent pleads that the shift was separated by only a 7-hour break, that this was a result of human error and not intentional or repeated, that in June 2020 the Appellant signed an agreement allowing an 8-hour break, and that "the appellant could refuse shifts at anytime". [¶¶ 226, 232 to 233] The Respondent's outlines of evidence served 24 September 2026 (Part B.6) give notice that its witnesses will say the break "was the result of human error", and that the advice returned within payroll was that "what had occurred was better characterised as a rostering practice issue for the line manager rather than an unpaid entitlement".

The role description states, under mandatory requirements, that the position is a continuous shift working role and that the occupant must be able to work a roster which covers multiple shifts over a 24/7 period. [¶ 13]

(h) The emergency workload on those shifts. Metro South Health has stated in writing that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024". [¶ 268] A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; and to support the allocation, coordination and fault repair of all pager units for the hospital. [¶¶ 7 to 8] The Switchboard staff are, on the employer's own statement, non-clinical. [¶ 265]

(h)(i) The codes. Each emergency code at Logan Hospital is received at the Switchboard, categorised by the operator by the type of emergency, and paged to the response group for that category within the time the procedure allows. [¶ 8] A Code Blue is the call for a medical emergency in which a patient has suffered, or is about to suffer, cardiac or respiratory arrest, and brings the resuscitation team. A MET call is the call for the Medical Emergency Team to a patient whose observations have deteriorated to the hospital's escalation threshold; a Neonatal MET call and a Paediatric MET call are the same call for a newborn and for a child. A Code Grey is the call for a security response to threatening or aggressive behaviour; a Code Black is the call for a security response to a personal threat. The colour codes follow the Australian Standard for emergency control in health care facilities, AS 4083. Queensland Health's own Code Blue procedure for the Royal Brisbane and Women's Hospital (Procedure 001443, version 5, effective April 2020) states that "there are eight different types of Code Blue", including Medical Emergency, Cardiac Arrest, Neonatal Emergency, and Paediatric Emergency or Paediatric Cardiac Arrest; that "Switchboard Services will activate one or multiple teams to provide a Code Blue response"; and that the response team "will be notified by a paging system from Switchboard Services... identifying the type of

Code Blue and the location of the emergency". The Logan Hospital emergency code procedures referred to in the role description [¶ 8] are the equivalent document for Logan Hospital (Part B.7.5).

(h)(ii) The codes on the shift of 17 March 2024. The shift ended at 23:00. [¶ 284] The Emergency Code Register for that day (Tab 31) records eight codes: at 3:57 a Code Grey in the ED lounge; at 5:19 a MET call to Ward 5A bed 27; at 7:48 a Code Grey in the ED corridor near short stay; at 13:48 a MET call to Ward 3B bed 2; at 14:13 a Code Grey in ED adult acute bed 15; at 19:16 a MET call to Ward 2H bed 7; at 19:20, four minutes later, a Code Blue to the same patient, Ward 2H bed 7; and at 21:16 a MET call to Ward 2Q bed 5. The last three fell in the evening of that shift.

(h)(iii) The codes on the shift of 18 March 2024. The shift began at 06:00, seven hours after the previous one ended. [¶ 284] The register records eleven codes that day: at 8:48 a Code Grey at the QAS triage airlock; at 9:20 a MET call to Ward 6A bed 8; at 11:31 a MET call to Ward 3DR bed 14, cancelled at 11:32; at 11:35 a Neonatal MET call to Theatre 2; at 13:09 a Neonatal MET call to Birth Suite 5; at 14:38 a MET call to Ward 3DR bed 11; at 14:45 and 14:55 Code Greys at QAS triage; and at 19:15 and 19:34 Code Greys on the ED QAS ramp. Six of those fell between 06:00 and 14:00: a Code Grey, three adult MET calls (one cancelled within a minute) and two Neonatal MET calls.

(h)(iv) The codes on 19 March 2024, the day of the sick leave. Sick leave of 7.60 hours was taken that day. [¶ 235] The register records sixteen codes: at 0:17 a MET call to Ward MIU bed 21; at 1:41 a Code Grey in ED ambulatory care; at 9:27 a Paediatric MET call to Ward CIU bed 16; at 10:28 a simulation MET call to Ward 7C bed 3; at 11:30 a MET call to Ward 2I bed 20 and at 11:34 a MET call to the same bed, annotated "CALLED VIA SWITCHBOARD" [sic]; at 12:36 a MET call to Ward 2I bed 12; at 13:42 a MET call to Ward 3DR bed 11; at 16:28 a MET call to Ward 3DR bed 11, cancelled at 16:29; at 16:48 a Code Grey in the ED mental health lounge; at 17:23 a MET call to Ward 3DR bed 11; at 17:31 a Code Grey in ED ambulatory care room 2; at 18:45 a MET call to Ward 5B bed 7; at 21:28 a Code Grey in the ED mental health lounge; and at 22:25 a Code Grey at ED QAS triage. Between 06:00 and 14:00 there were six: a Paediatric MET call, a simulation, and four adult MET calls, one of them recorded as called through the Switchboard.

The entries are those recorded in the register at Tab 31. The propositions drawn from it (paragraphs 228 to 231 of the notice: the eight entries on 17 March including the MET call and Code Blue to Ward 2H bed 7; the six entries between 06:00 and 14:00 on 18 March including the two Neonatal MET calls; and the 19 March entry "CALLED VIA SWITCHBOARD") were not admitted, and the authenticity of the copy is in issue (Part B.7.3). The employer holds its own record of the MET calls for 17 and 18 March 2024. [¶ 268]

(h)(v) The ordinary call load, and the Monday. In April 2025, during the Appellant's graduated return, the console took between 269 and 444 calls on a single eight-hour shift, an average of about 360, on the monthly call statistics for that period; the highest figure, 444, was the one Monday afternoon shift in that period, 14 April 2025. On the Appellant's account the Monday early shift is the peak of the week: the weekend's backlog, the resumption of clinics and elective lists and the Monday admissions arrive at the console together. The shift of 18 March 2024 was the Monday early shift, the first shift of the working week [¶ 227], and it followed a break of seven hours. [¶ 284]

(i) The leave of 19 March 2024. The Leave Takings Report produced by Metro South Health in respect of 19 March 2024 records the Leave Category as "Sick", the Leave Type as "Sick Leave", the Time Code as "SCK", the Leave Taken as 7.60 hours and the Leave Status as approved. [¶ 235]

The Respondent pleads that the Appellant took leave on 19 March 2024 but says this was paid leave, and that pursuant to clause 18.10 of the Award he is not entitled to fatigue leave because he was not performing overtime. [¶¶ 236 to 237] The Respondent's outlines (Part B.6) give notice that its payroll witness will say the Award's fatigue provision "applies to breaks around overtime, not between two ordinary shifts". On that account, no entitlement to recovery time arose from a seven-hour break between two ordinary shifts. The only protection was the roster itself.

(j) The enquiry of 8 April 2024, the delay, and the refusal of 1 May 2024. The review decision records: "You emailed Ms Taylor on 8 April 2024 and requested a review of your payment for the pay period between 8 March 2024 and 31 March 2024", noting the Award requirement as to a minimum ten-hour break. [¶¶ 242 to 243]

It records that she replied on 9 April 2024 and "added she also escalated your enquiry regarding fatigue leave to Human Resources to confirm policies around this", and that she "proceeded to acknowledge the importance of communication and requested that you raise your concerns with her as soon as they arise

so she could action them sooner for you". [¶¶ 244 to 245]

It records that on 24 April 2024 the Appellant emailed expressing dissatisfaction that it had been more than two weeks without any response or overtime payment. [¶ 246]

The Respondent does not allege that any response was made to the fatigue enquiry between 9 April and 1 May 2024; the enquiry of 8 April was answered after 23 days. [¶¶ 249 to 250]

It records that she responded on 1 May 2024 stating that after a consultation with payroll and Human Resources the request for fatigue payment for 18 March 2024 would not be processed due to the existing 8-hour agreement, and noting that he was able to terminate that agreement going forward. [¶¶ 247 to 248]

It records that payroll was, as at 1 May 2024, still reviewing his entitlements; that the employer did not mention his shift on 18 March 2024; and that there was uncertainty between him and the employer regarding whether the 8-hour agreement continued to apply. [¶¶ 253 to 255]

So the enquiry of 8 April 2024 was answered on 1 May 2024, 23 days later, with a refusal. In between were the acknowledgment of 9 April, the escalation to Human Resources, and the Appellant's email of 24 April; no other response is alleged. [¶¶ 242 to 250]

(k) The employer's own fatigue policy. The review decision records that the policy regarding fatigue leave following weekends and rostered days off is dated June 2020. [¶ 239]

It records that "The policy notes that many problems can be overcome regarding fatigue leave by either not rostering the employee who is on call for the first shift of the following day and not rostering on call an officer who is rostered for the first shift on the following day." [¶ 240]

(l) The absence of fatigue risk management at the Switchboard. By letter of 5 June 2026, reference K-LM26/729, signed by the Chief Executive and addressed to the Commission, Metro South Health stated that the requested fatigue risk management documents "do not exist", that mandatory Fatigue Risk Management System training "only applies to health practitioners and clinical assistants" and that Switchboard staff are non-clinical. [¶¶ 263, 265] It stated that "The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024". [¶ 264] It stated that "there have been no 'consequential' changes to operating procedures over the period requested". [¶ 266] The Respondent does not allege that any fatigue risk assessment was conducted, that fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024. [¶¶ 269 to 272]

(l)(i) The employer's own fatigue guideline. On 20 May 2024 the Director sent Human Resources the Queensland Health Fatigue Risk Management Systems Implementation Guideline, qh-gdl-401-3.3. [¶¶ 222 to 223] Ten days earlier she had written that she and the Manager had run through the Roster Risk Assessment Matrix and "we say at best there would be a rating of 11 which is moderate". [¶ 219] The Respondent disclosed the guideline, with that email chain, on 14 July 2025 (Tab 8A). The following passages are from that disclosed copy.

The guideline defines fatigue as "A state of impaired physical and/or mental performance and lowered alertness arising as a result or combination of physical and mental work, health and psychosocial factors or inadequate restorative sleep", and a fatigue risk management system as "An integrated set of management practices and procedures for monitoring and managing the risks posed to health, safety and wellbeing by fatigue". It defines "Defences in depth" as a "Hazard identification and risk control model that applies a series of layered mechanisms to minimise the occurrence of fatigue related incidents".

[Guideline, glossary, p 29]

It provides that "Level 4 controls relate to error tolerance and containment" and that "Error tolerance relates to built-in redundancies that decrease the likelihood that a fatigued individual will make an error which may have catastrophic consequences." [p 21] It lists among its related documents the Hospital and Health Services General Employees (Queensland Health) Award - State 2015 and the "I1 (QH-POL-171) Fatigue risk management policy". [p 27, section 9]

On consultation and training, the guideline states: "there is a legislative obligation to consult with workers in relation to hazard identification and the making of decisions about how associated risks can be managed. Consultation requirements for Queensland Health are described in Health, safety and wellbeing consultation standard (QH-IMP-401-2:2020)." It states that information, training and instruction "must have regard for: the nature of the work carried out; the nature of the risks associated with the work; the control

measures implemented", and that "where a work unit or an occupational stream has been identified at risk of fatigue, the training provided must consider the above criteria". Its example training matrix (Figure 11) lists, as those to be trained, "Staff at risk of fatigue", "Rostering staff" and "Manager, supervisors, executives", with learning outcomes including "Rostering". [p 26, section 7.1]

Its supporting documents include the "Fatigue risk assessment tool", the "Individual fatigue likelihood calculator" and the "Samn-Perelli fatigue checklist". [pp 27 to 28, section 9] It defines a "Worker" by reference to section 7 of the WHS Act, including "An employee", and states that "The Department of Health and each of the HHSs are considered to be PCBU's". [pp 29 to 30, section 10] Its version control records that version 1.0 of 10 November 2021 replaced the "Fatigue Risk Management System Resource Pack December 2018 v. 3". [p 30, section 11] Its references include "Queensland Ombudsman (2006). The Neville Report". [p 31, section 12]

So the guideline treated the Award under which the Appellant was employed as a supporting document. It treated every employee of a Hospital and Health Service as a worker to whom it applied. It called for consultation with workers on fatigue hazards, and for training of rostering staff and managers. A resource pack had preceded it since December 2018. The Chief Executive's letter states that fatigue risk management assessment at the Switchboard was implemented after 30 June 2024 and that the mandatory training "only applies to health practitioners and clinical assistants". [¶¶ 264 to 265] The Respondent does not allege that any fatigue risk management training was provided to the Appellant, or that any fatigue risk assessment of his position was conducted, before 30 June 2024. [¶¶ 269 to 270]

The guideline the Director sent to Human Resources on 20 May 2024 is the guideline the Chief Executive's letter of 5 June 2026 says was implemented at the Switchboard only "after 30 June 2024" [¶ 264], and the training for which "only applies to health practitioners and clinical assistants" [¶ 265]. The fatigue risk of the Appellant's roster had been assessed by the Director and the Manager as moderate on 10 May 2024 [¶ 219]; no fatigue risk assessment of his position is alleged to have been conducted before 30 June 2024 [¶ 269].

(l)(ii) Where the framework came from. The guideline's references list "Queensland Ombudsman (2006). The Neville Report". [p 31, section 12] That report followed the death of Elise Neville, aged 10, after her presentation to the Caloundra Hospital emergency department in January 2002, where the only doctor on duty was "in the 20th hour of a continuous shift". The Ombudsman identified "unsafe working hours for junior doctors" and "rostering practices that required excessive hours of work" among the systemic flaws found, wrote that "anyone who is in the 20th hour of a continuous shift must have reduced capacity to assess the situation when it presents itself", and recommended that Queensland Health determine "an interim standard on safe working hours for doctors in public hospitals" and implement "management practices aimed at alleviating the ill-effects of excessive working hours" (Recommendations 4 and 5). Queensland Health's Policy on Medical Fatigue Risk Management took effect on 1 July 2008 and required each health service district to "develop, implement and maintain a fatigue risk management system" for medical officers (Queensland Ombudsman, The Neville Report Update, 2011). The resource pack of December 2018 and the 2021 guideline that replaced it apply the system to every "worker" of a Hospital and Health Service, including "An employee". [pp 29 to 30, section 10; p 30, section 11] The Chief Executive's letter of 5 June 2026 states that the mandatory training "only applies to health practitioners and clinical assistants" and that assessment at the Switchboard was implemented after 30 June 2024. [¶¶ 264 to 265] The Switchboard is where the codes are received and paged. [¶ 8]

(m) The findings in the decision under appeal. The review decision states: "The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours." [¶ 258]

It states that even if the Appellant was allowed to leave early on 17 March 2024 as suggested by the employer, he left a maximum of thirty minutes early, which meant he still did not receive a minimum 8-hour break. [¶ 259]

It states: "Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement." [¶ 260]

It states, under the heading "Conclusion", that the Appellant "sustained a personal injury of a psychological nature". [¶ 261]

It states that his injury "arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor". [¶ 262]

(n) The Respondent's pleaded answer to this stressor. Beyond the matters at (g) and (i) above, the Respondent does not allege that the Appellant was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024, nor does it describe any communication with him before that date as a warning. [¶¶ 300 to 302] Paragraph 27 of its amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. [¶¶ 238, 303] The employer's own account of its complaint handling is that complaints "are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant", and no change to operating procedures as a consequence of any employee complaint is alleged. [¶¶ 267, 272]

The medical link - Stressor 3. Tab M1 records the 1 July 2024 report that the roster was being manipulated so the Appellant worked "lates then earlies". Tab M4 later records workplace stress involving management and rostering. Tab M8 is the review decision recording the seven-hour break and the finding that the rostering of the 17 and 18 March shifts was unreasonable management action. Tab M7 is relied on only for continuing effect and capacity in 2026. The medical material is not relied on to prove the roster events. Those events are proved by the admitted facts above about the roster, the policy, the correspondence and the review decision.

The medical case as a whole. The medical material does not give a separate diagnosis for each pleaded event. The Appellant relies on one psychological injury arising from the whole course of conduct. Repeating an admitted fact under more than one stressor therefore serves a purpose and is not double counting. It allows each stressor to be decided on its own, while keeping the whole course of conduct that was described to the treating doctors.

3. What happened after the injury, and the record made at the time

3.1 After the injury - context only, not cause. The work capacity certificates certify no functional capacity from 1 July to 6 October 2024. Matters after the date of injury are not relied on as causes of it and are not pleaded.

3.2 What the Appellant provided to WorkCover Queensland, and when. The Respondent's amended List of Documents of 14 August 2026 records that the Appellant provided to WorkCover Queensland, at item 12, an email of 12 July 2024 with an attachment described as "Event overview - undated"; at item 14, an email of 18 July 2024 with an attachment described as "Witness statement - Carolyn Jeffrey"; at item 25, an email of 29 August 2024 described as regarding the after hours on call change, with an attachment described as "Email: After hours on call process"; at item 26, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: Task change switchboard - 19/04/2024"; and at item 27, an email of 30 August 2024 described as regarding failure to consult, with an attachment described as "Email: MASPER process - 09/05/2024". It further records, at item 16, an email from Ms Carolyn Jeffrey to WorkCover Queensland of 1 August 2024 described as a follow up statement. [¶¶ 274 to 280] The documents at items 25, 26 and 27 are the three directives pleaded at Stressor 1(e) and Stressor 1(g) above. They were provided by the Appellant to WorkCover Queensland in August 2024, before any decision on his claim and more than two years before this pleading.

3.3 The particulars bundle of 11 August 2026. The Respondent's amended statement of facts and contentions states at paragraph 11 that it does not admit the allegations in Stressor 1(a) of the Appellant's statement "because there are no particulars or details to respond to". On 11 August 2026 the Appellant served a bundle titled "Stressor 1(a) - Particulars support bundle", comprising 30 pages and six tabs, each stating a particular of that stressor and enclosing the documents recording it. [¶¶ 281 to 282]

4. The intervals

The intervals below are worked out from dates and times admitted by the Respondent. They are arithmetic only. They do not depend on whether any statement in a document is true. Each is pleaded in its place in Part B.2; this table collects them.

Interval	Length	Where pleaded	Facts
Rostered finish on 17 March 2024 at 23:00 to rostered start on 18 March 2024 at 06:00; the minimum is ten hours, or eight by written agreement, and the agreement applied only to staff-initiated swaps, none being alleged; with travel, less than five hours	7 hours	Stressor 3(d) to (g); the Review Unit's own finding at 3(m)	¶¶ 284 to 285, 225, 234, 251, 260
MASPER Registrar's first report, 3 May 2024 at 3:06 pm, to the first reply, 9 May 2024 at 9:20 am (her second report was sent on the fifth calendar day; the reply on the sixth)	5 days, 18 hours, 14 minutes	Stressor 1(g)	¶¶ 62 to 64
Integrated Respiratory Service's first email, 15 May 2024 at 11:47 am, to its second, 20 May 2024 at 11:03 am	the fifth calendar day	Stressor 1(h)	¶ 92
That first email to the Appellant's escalation, 20 May 2024 at 2:05 pm	5 days, 2 hours, 18 minutes	Stressor 1(h)	¶¶ 97 to 98
The Service's second email to that escalation	3 hours, 2 minutes	Stressor 1(h)	¶ 97
The escalation to the reply at 4:30 pm, two hours after the office hours stated to all Switchboard staff on 17 May 2024 had ended	2 hours, 25 minutes	Stressor 1(h)	¶¶ 102 to 103
The Appellant's fatigue enquiry, 8 April 2024, to the response, 1 May 2024; no response alleged in between	23 days	Stressor 3(j)	¶¶ 249 to 250
Third submission of the special pandemic leave request, 29 February 2024 at 11:07:28, to its approval at 11:21:03	13 minutes, 35 seconds	Stressor 1(l), 2(f)	¶ 124
Creation of the draft, 20 February 2024, to final approval, 1 March 2024	10 days, 4 hours, 40 minutes, 20 seconds	Stressor 1(l), 2(f)	¶ 125
Removal of database access, 18 July 2023, to 18 June 2024; no restoration alleged	11 months	Stressor 1(b)	¶ 55
23 August 2023 to 17 May 2024; no fixed office hours alleged to have been stated	9 months	Stressor 1(i)	¶ 73
Payroll's direction, 3 May 2024, to the pay date of the payslip carrying three of the four corrections, 5 June 2024	33 days	Stressor 2(e)(i)	Payroll disclosure, July 2025
That direction to the pay date of the payslip carrying the fourth, 2 July 2025 (amendment dated 30 June 2025)	425 days	Stressor 2(e)(i)	Payroll disclosure, July 2025
The Appellant's last shift, 3 June 2024, to the first of those pay dates	2 days	Stressor 2(e)(i); Part B.1.4	-
The pay enquiry of 8 April 2024 to the reply of 9 April	1 day	Stressor 2(e)(i), 3(j)	¶¶ 242, 244
That reply to the Appellant's follow-up of 24 April 2024	15 days	Stressor 2(e)(i), 3(j)	¶¶ 244, 246
That follow-up to the refusal of 1 May 2024	7 days	Stressor 2(e)(i), 3(j)	¶¶ 246, 247
The refusal of 1 May 2024 to Payroll's direction of 3 May 2024	2 days	Stressor 2(e)(i)	¶¶ 247, 185
Payroll's direction to the Appellant's email to Payroll, 10 May 2024	7 days	Stressor 2(e)(i)	¶¶ 185, 191
Payroll's direction to Payroll's email that nothing had been corrected, 13 May 2024	10 days	Stressor 2(e)(i)	¶¶ 185, 193
Payroll's direction to the Manager's "still waiting" email, 21 May 2024	18 days	Stressor 2(e)(i)	¶¶ 185, 195
Payroll's direction to the Manager's AVAC of 28 May 2024	25 days	Stressor 2(e)(i)	¶¶ 185, 197, 209
That AVAC to its processing as "Part Completed", 30 May 2024	2 days	Stressor 2(e)(i)	¶ 203
The shifts of 28 February, 30 March and 1 April 2024 to the payslip that adjusted them, pay date 5 June 2024	98 days; 67 days; 65 days	Stressor 2(e)(i)	¶¶ 187 to 189; Payroll disclosure, July 2025
The shift of 9 February 2024 to the payslip carrying its correction, pay date 2 July 2025	509 days	Stressor 2(e)(i)	¶¶ 185 to 186; Payroll disclosure, July 2025

Interval	Length	Where pleaded	Facts
That request to the hours being stated to the department, 17 May 2024 at 9:30 am	1 day, 20 hours, 15 minutes	Stressor 1(i)	¶¶ 74, 81
That request to the Director's question about which changes had concerned him, 21 May 2024 at 2:53 pm	6 days, 1 hour, 38 minutes	Stressor 1(i)	¶¶ 74, 79

Particulars: paragraphs 55, 62 to 64, 73 to 74, 79, 81, 92, 97 to 98, 102 to 103, 124 to 125, 185 to 197, 203, 209, 225, 234, 242 to 247, 249 to 251, 260, 284 to 285 of the notice to admit facts served 28 August 2026, admitted 8 September 2026; the Respondent's payroll disclosure of July 2025 (Part B.7.5).

5. Matters the Respondent does not allege, and documents it does not list

The Respondent does not allege any of the matters listed under the headings below. It has admitted this as to the state of its amended statement of facts and contentions of 13 May 2026 and its amended List of Documents of 14 August 2026. These are admissions about what those documents do and do not contain. They are not admissions that the contents of any document are true, and the qualification in the Respondent's letter of 8 September 2026 does not apply to them.

5.1 As to the directory and the changes to process. That access to the database was restored to the Appellant before 18 June 2024; that any document records consultation with Switchboard operators before the change communicated on 15 April 2024; that agreement under clause 6.2 of the Award was obtained, or a ballot conducted, before that change.

5.2 As to what followed the reports at the console. That any communication was sent to Logan Switch or the Switchboard staff about the occasions reported on 3 and 8 May 2024 before 10:15 am on 9 May 2024; that any response was made to the Integrated Respiratory Service before 20 May 2024, or before 2:05 pm on that day; that any document created on 20 May 2024 records the discussion referred to in the reply of that afternoon; that any communication was sent by either named person to that Service on that day; that the contact-details document was amended on or before 20 May 2024; that the Switchboard staff were notified of the modifications made to it on 22 February 2024; or that the Appellant was rostered to work after 2:05 pm on 20 May 2024.

5.3 As to the office hours. That fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024.

5.4 As to the Communication Book. That the entry removed, or its author, is identified; that the removed page has been located or that any copy of it exists; that any entry made by the Appellant remains in the book; or that the book ceased to be in the possession of Metro South Health during the Appellant's employment. No proposition about whether the book is listed in the amended List of Documents is relied upon in this paragraph; fact 154 was not admitted.

5.5 As to the special pandemic leave. That any other Switchboard employee was required to submit such a request personally through myHR in February 2024.

5.6 As to the roster and fatigue. That the Switchboard Manager got in touch with the Appellant about alternative shifts after the meeting of 16 April 2024; that the fatigue toolkit was reviewed or feedback given before 18 June 2024; that the consecutive shifts of 17 and 18 March 2024 arose from a staff-initiated shift swap; that any response was made to the fatigue enquiry between 9 April and 1 May 2024; that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, or that fatigue risk management assessment was implemented at the Switchboard, before 30 June 2024; or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint over the period 1 December 2023 to 30 June 2024.

5.7 As to the Appellant's performance and conduct. That he was subject to any disciplinary process, or that his work performance was the subject of any formal performance management process, before 18 June 2024; nor does the Respondent describe any communication with him before that date as a warning. Paragraph 27 of its statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon for the contention in that paragraph.

Particulars: paragraphs 16, 21 to 22, 25, 55, 69, 73, 90, 93, 99, 105 to 110, 142, 150 to 153, 166, 180 to 181, 213, 216, 234, 250, 269 to 273, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026. Paragraph 154 of that notice is not admitted.

6. The Respondent's notified evidence, served 24 September 2026

On 24 September 2026 the Respondent served outlines of the evidence of Ms Taylor, Ms Reese, Ms Wright and Ms Earl. An outline is not evidence. The statements below are set out as notice of the Respondent's case, so that the issues are defined before the hearing. They are relied on only as the Respondent's own description of the matters they concern. The Appellant will ask each witness to confirm them.

6.1 Ms Taylor. The outline states that she "was the Switchboard Manager, Logan Hospital, and the Appellant's direct line manager, in an acting capacity, from around April 2023 to 1 July 2024 supervising rostering, attendance and shifts"; that she "Confirms the seven-hour break between shifts on 17-18 March 2024 was the result of human error"; that she "Confirms the AVAC correspondence of 3, 21 and 28 May 2024, in which she was involved in emails about correcting the Appellant's shift payments"; that she "sought advice from HR, who confirmed the agreement was on file and continued to apply", no document recording that advice being identified; and that, acting on HR's advice, she "removed the email from the shared switchboard inbox herself", being the email of 15 May 2024 at Stressor 1(i).

6.2 Ms Reese. The outline states that the email of 15 May 2024 was "asking that Ms Taylor's office hours be shared with the whole department"; that she emailed the Appellant "at 4:12pm" and "again in materially the same terms at 6:23pm" asking him to retract it, on the basis that it "could have been raised with Ms Taylor or Ms Reese directly rather than sent to the whole team"; that on 7 August 2023 the Appellant raised "concerns about rostering fairness" and "a claimed breach of the enterprise agreement's roster-display requirements", and that she "advised him of his option to lodge a formal grievance with HR"; and that her understanding that the eight-hour agreement continued to apply after the move to full-time employment was "through Ms Taylor".

6.3 Ms Wright (Payroll). The outline states that the pay issues raised "were investigated and, where a discrepancy or error was found, corrected"; that a public holiday allowance "must be approved and submitted by the line manager on an AVAC"; that the agreement of 17 June 2020 reduced "his break between shifts from ten hours to eight"; that payroll "does not itself assess an employee's entitlement under them, or under any change in employment status"; and that, when the system did not apply an additional payment for the break of 17 to 18 March 2024, the advice returned was "that the Award's fatigue provision applies to breaks around overtime, not between two ordinary shifts, and that what had occurred was better characterised as a rostering practice issue for the line manager rather than an unpaid entitlement".

6.4 Ms Earl (Payroll). The outline states that the Appellant "uploaded his Special Pandemic Leave application together with a supporting attachment on 20 February 2024 at 11:41am"; that it was declined on 21 February "on the stated basis that a minimum five-day period of leave was required" and on 29 February "for want of an attached statutory declaration or evidence of a positive test"; that "On payroll's own review of the system's attachment history, however, the supporting document had in fact been attached from the original 20 February submission onward, meaning the second decline also appears to have been made in error"; that it was then "approved that day by the delegate and manager" and that the Appellant "continued to be paid, throughout the period this was being sorted out"; that for the fortnight including 30 March 2024 the public holiday was first processed as "public holiday not required", which "triggered an automatic hours reduction on his payslip", and that "The entry was later corrected"; and that from the roster the break of 17 to 18 March 2024 "was only seven hours, that he was rostered on ordinary hours rather than overtime at the time, and that he took paid sick leave on 19 March 2024".

6.5 Matters the outlines do not address. No outline addresses the removal of database access or its restoration; the Contact & Number Changes book; any consultation preceding the change notified on 15 April 2024; the reports of the MASPER Registrar of 3 and 8 May 2024; the emails of the Integrated Respiratory Service of 15 and 20 May 2024; the amendment of the clinic contact-details document; the fatigue enquiry of 8 April 2024 or the interval to 1 May 2024; the email to Human Resources of 10 May 2024; or the absence of fatigue risk management at the Switchboard before 30 June 2024. No medical or expert evidence is notified. The Respondent's list of witnesses, sent to the Registry for filing and to the Appellant on 24 September 2026, names those four witnesses, described as "lay" witnesses, and no other, and states that the Respondent "reserves its right to amend this list depending on the case presented by the appellant at the hearing".

7. The documents at Annexure A, and their status

The 39 documents at Annexure A to the notices to admit served 28 August 2026 are the documents from which the facts in Part B are drawn. On 8 September 2026 the Respondent admitted the authenticity of 25 of them and disputed 14. By email of 23 September 2026 it confirmed four of the 14 "upon further review of the appeal file", stated that it had "requested copies of these documents to be disclosed from MSH" for nine others, and did not mention the fourteenth. The facts recorded in 13 of the 14 were admitted on 8 September 2026 to the extent set out in Part B; the exception is the register at Tab 31, from which facts 228 to 231 were not admitted. For the other 13, what remains in dispute is the authenticity of the copies, not their content.

7.1 Admitted on 8 September 2026 (25 documents). Tab 1A, Ms Stibbard to the Switchboard team, "Hello & Update", 18 July 2023, 12:56 pm. Tab 1B, Ms Taylor to Ms Reese, "Fwd: Communication Book Update", 6 June 2023, 4:05 pm, with her email to Logan Switch of 9:57 am that day. Tab 2, the Appellant's email chain "Increase of hours and Workplace issues" and Ms Reese's replies, 7 to 10 August 2023. Tab 3, Ms Reese's email attaching HR Policy E12 and the emails of 4 and 8 September 2023, 29 August to 8 September 2023. Tab 4, "Request to Increase Working Hours to Full Time Rotational Roster" and the emails of 7 August 2023 at 1:43 pm and 5:11 pm, 7 and 31 August 2023. Tab 7, the "Roster Concerns" chain, 26 April to 8 May 2024. Tab 8, Ms Reese to Mr Pritchard and to LBH_HR, 10 and 20 May 2024. Tab 8A, the cover page of QH-GDL-401-3.3:2021. Tab 8B, the MASPER emails and issue logs, 2 to 9 May 2024. Tab 8C, Ms Stibbard's on-call email, 13 May 2024, 4:29 pm. Tab 9, "Switchboard Manager - On call and Hours.", 17 May 2024, 9:30 am. Tab 9A, the "Office Hours and Departmental Directives" chain, 15 May 2024, 1:15 pm, 6:23 pm and 7:09 pm. Tab 9B, Ms Reese's reply, 21 May 2024, 2:53 pm. Tab 10, "Sick leave 14.05.24", 14 May 2024, 12:08 pm, and its forwarding. Tab 11, the Integrated Respiratory Service emails and the replies, 15 and 20 May 2024. Tab 12, "Corey Shepherd 388372 Pay issues", 3 to 13 May 2024. Tab 13, the same thread, 21 May 2024, 12:33 pm. Tab 14, "Validation of Claims older than 3 months - Please sign", 28 May 2024, 8:36 am. Tab 15, the myHR submissions report, 1 February to 31 May 2024. Tab 16, the Leave Takings Report, 19 March 2024. Tab 25, Review Decision 69983, 24 October 2024. Tab 26, the Appellant's first notice to admit facts. Tab 27, the Respondent's response of 18 February 2026. Tab 28, the myHR leave request history for 15480560, 20 February to 1 March 2024. Tab 29, the Instrument of Human Resource Sub-Delegation, effective 5 December 2022.

7.2 Disputed on 8 September 2026, confirmed by the Respondent on 23 September 2026 (4 documents). Tab 6, Ms Taylor to Logan Switch and the Switchboard staff, "Afterhours Oncall Process - Switchboard", 15 April 2024, 12:39 pm (Stressor 1(e)). Tab 20, the letter of Metro South Hospital and Health Service to Commissioner Dwyer, reference K-LM26/729, 5 June 2026 (Stressors 1(m) and 3(l)). Tab 30, Ms Taylor to Logan Switch and the Switchboard staff, "What's Chloe's Hours?!", 23 August 2023, 2:18 pm (Stressor 1(i)). Tab 30A, Ms Taylor to Logan Switch, "Good morning Team.", 18 June 2024, 8:58 am (Stressor 1(i)). Each was confirmed from the Respondent's own appeal file.

7.3 Disputed on 8 September 2026 and referred by the Respondent to Metro South Health on 23 September 2026 (9 documents). Tab 1, the role description, Administration Officer, Switchboard Services (AO3), Logan Hospital, undated; its terms are admitted at facts 1 to 13. Tab 5, Ms Taylor to the Appellant, "Approved - Permanent Full Time FTE", 27 September 2023, 1:52 pm; its terms are admitted at facts 37 to 38. Tabs 17, 18 and 19, the movement forms approved by Mr Hughes as delegate, 27 February, 17 April and 9 June 2026, each recording "Continuous Shift Worker"; admitted at facts 14 to 15. Tab 21, Ms Forrest, Senior Consultant, Human Resources, to the Appellant, 7 July 2026, stating that the 8-hour agreement "is only applied where staff initiated shift swaps have occurred"; admitted at facts 224 to 225. Tab 22, the Consultation Paper, Proposed Rosters for Switchboard Services, Logan Hospital, November 2024, signed by Mr Hughes on 12 November 2024; admitted at facts 169 to 175. Tab 23, the Consultation outcome, December 2024, signed by Mr Hughes on 12 December 2024; admitted at facts 167 to 168 and 176 to 177. Tab 31, the screen capture of the workbook "2024 Emergency Code Register.xlsx", March 2024 sheet, entries 16 to 20 March 2024 (Stressor 3(h)); facts 228 to 231 were not admitted. Each of these nine is a Metro South Health document, and five of them (Tabs 17 to 19, 22 and 23) bear the signature of its Director, Corporate Services. The Respondent's email of 23 September 2026 gives no date by which the copies are expected.

7.4 Disputed on 8 September 2026 and not addressed on 23 September 2026 (1 document). Tab 24, Mr Moran, Organiser, Together Queensland, to Ms Jeffrey, Ms Conaghan and the Appellant, "Switchboard Roster Feedback - For Delegates", 3 November 2025; its terms are admitted at facts 178 to 179. It is not a

Metro South Health document, and the Respondent's email of 23 September 2026 does not refer to it.

7.5 Documents not at Annexure A that the Respondent's own List of Documents or letter identifies.

The spreadsheet of recorded MET calls for 17 and 18 March 2024, which Metro South Health states is available [¶ 268]. The Logan Hospital emergency code procedures referred to in the role description [¶ 8]. The documents recording the advice of Human Resources on the 8-hour agreement, to which the Respondent's outlines of Ms Taylor and Ms Reese refer (Part B.6) and which neither identifies. The Communication Book, which the Respondent's List of Documents does not list; paragraph 154 of the notice was not admitted. The Respondent's disclosure of July 2025 headed "Disclosure from witness conferencing - Qld Health Payroll" (24 pages): the email of Ms Christensen, Manager Service Delivery, Payroll, to the Respondent of 11 July 2025, and Payslips 01 to 07, including Payslip 06 (pay date 5 June 2024) and Payslip 07 (pay date 2 July 2025), relied upon at Stressor 2(e)(i). The Respondent's disclosure of 11 June 2025 (10 pages): the leave-form walkthrough for Process Reference 15480560, the emails of 15 May 2024 at 1:15 pm and 6:23 pm, the emails of 15 and 16 May 2024 concerning the complaint of 13 May 2024, and the complaint form; relied on at Stressor 1(n) for dates, senders and recipients only. Public documents relied on at Stressor 3: Queensland Health's Code Blue procedure 001443 (Royal Brisbane and Women's Hospital, version 5, April 2020), at 3(h)(i) for the types of Code Blue and the role of Switchboard Services; the Queensland Ombudsman's Neville Report (June 2006) and its 2011 update, and the Coroner's findings of 12 September 2008, at 3(l)(ii) for the origin of the fatigue framework; the monthly call statistics for April 2025, at 3(h)(v).

PART C - CONTENTIONS

1. The onus of proof, accepted. The Appellant accepts that it is for him to prove, on the balance of probabilities, that he sustained an injury within section 32, and that the exclusion in section 32(5)(a) does not apply to the injury proved. Whether management action was reasonable is judged objectively. *Prizeman v Q-COMP* [2005] QIC 53; (2005) 180 QGIG 481 states that the focus is on what the employer actually did, not on how the worker perceived it. The contrary contention in the Amended Form 9A filed 7 April 2026 is withdrawn.

2. The admitted record is the factual basis for that assessment. The Appellant does not ask the Commission to find conduct unreasonable from labels alone. The Respondent has admitted, to the extent recorded in its response of 8 September 2026, the facts drawn from the documents set out in Part B. They include: that database access was removed and is not alleged to have been restored before 18 June 2024; that no document records consultation with Switchboard operators before the 15 April 2024 change; that the first report of calls reaching the wrong medical team came nearly six days before the first identified communication to Switchboard staff about them; that the record of 20 May 2024 contains the gaps identified at Stressor 1(h); and that the employer's letter of 5 June 2026 states that the identified fatigue records do not exist, that the mandatory training applied only to health practitioners and clinical assistants, that fatigue assessment at the Switchboard began after 30 June 2024, and that there were no consequential changes to operating procedures over the period requested.

3. Injury under section 32(1). The Appellant contends that he sustained a personal injury of a psychological nature arising out of, or in the course of, his employment, with employment a significant contributing factor. The injury is the diagnosed condition identified in Part A. It is pleaded as arising from the whole course of conduct in Part B.2, not from an event on a single day. The Respondent's review decision recorded both a psychological injury and employment as a significant contributing factor, while rejecting the claim under section 32(5).

4. Section 32(5)(a) - working conditions that are not management action. The Appellant contends that some of the circumstances in Stressor 1 are conditions in which the work had to be done, not management action. They are: the position stayed responsible for accurate contact and paging information while operators could not directly amend the database; calls, including a MET-call enquiry, were sent to the wrong medical team; a clinical service reported that it could not help patients or staff because of a wrong directory entry; and operational complaints were, on the employer's own account, handled in the way recorded in its letter of 5 June 2026. The Commission is asked first to decide which circumstances are management action and which are not, and then to apply the exclusion.

5. Section 32(5)(a) - if management action, whether it was reasonable and taken in a reasonable way. To the extent the pleaded circumstances are management action, the Appellant relies on the admitted sequence, timing and gaps in the documents in Part B. In particular: the 15 April process change was effective on the day it was notified, and no document of prior consultation is listed; pay corrections for the identified fortnights were sent back through the line-manager process, one later claim is still recorded as "Part Completed", and two identified February AVAC references do not appear in the report; the paid Special Pandemic Leave entitlement in Stressor 2(f) took three submissions and was declined twice, and the Respondent later pleaded that the supporting attachments were in fact present when the later decline was made; the fatigue enquiry was escalated to Human Resources and answered after 23 days, with no response alleged in between; and the roster decision described in the email of 10 May 2024 was not to be disclosed to the Appellant until the implementation plan was issued. The Appellant relies on the intervals in Part B.4, and on the matters the Respondent does not allege and the documents it does not list in Part B.5.

5A. The Respondent's notified evidence is consistent with the admitted record. On the Respondent's own outlines (Part B.6): the seven-hour break was "the result of human error" and, on the advice returned within payroll, "a rostering practice issue for the line manager"; the agreement of 17 June 2020 reduced the break "from ten hours to eight", so that a seven-hour break fell below both; no recovery entitlement attached to a short break between two ordinary shifts; the second decline of the paid Special Pandemic Leave request "appears to have been made in error"; and pay corrections were the line manager's to submit. The Appellant contends that the rostering of 17 to 18 March 2024 was, on the Respondent's own description, an act of the line manager in rostering; that it is management action; and that it was not reasonable management action taken in a reasonable way, for the reasons in Stressor 3. The matters the

outlines do not address (Part B.6.5) stand on the admitted facts.

6. Assessment as a whole, where appropriate. *Delaney v Q-COMP Review Unit* [2005] QIC 11; (2005) 178 QGIG 197 supports assessing management action as a whole where its parts are closely enough connected by subject matter, time and the people involved. The Appellant relies on that approach only where the Commission considers the connection close enough. Each stressor is also pleaded on its own, so that the reasonableness question can be answered without bringing in a fact from another stressor.

7. The ultimate statutory question concerns the injury as a whole. *Workers' Compensation Regulator v Langerak* [2020] ICQ 2, as applied in *Carr v Workers' Compensation Regulator* [2022] QIRC 59 at [60], states that the ultimate question under section 32(5)(a) is whether the injury, not any one stressor, arose out of or in the course of reasonable management action taken in a reasonable way. That requires the reasonable management action to be weighed against the other employment factors, in light of the whole of the evidence. *Simon Blackwood (Workers' Compensation Regulator) v Mahaffey* [2016] ICQ 10 is consistent with the proposition that, where other employment factors also caused the disorder, the presence of reasonable management action among the causes does not by itself answer the statutory question. The review decision's finding that the rostering of 17 and 18 March 2024 was unreasonable management action is relied on as an admitted document, not as a binding decision.

8. The effect of the admissions. The admissions are made for this proceeding under rule 49. They are relied on as evidence that the documents admitted exist and of what they say, and as evidence of what the Respondent's statement of facts and contentions and List of Documents do not contain. The Appellant does not contend that any admission decides whether any conduct was reasonable; that is for the Commission.

9. The hearing de novo. The Respondent admits the contents of Review Decision 69983 and says no finding in it binds the Commission because the hearing is de novo (a fresh hearing). [¶ 295] That is accepted, and it applies to the whole of the decision. The conclusion under appeal is a conclusion of that decision, reached on the findings recorded in it.

10. The Respondent's pleaded case. Paragraph 27 of the amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. No disciplinary process, formal performance management or communication described as a warning before 18 June 2024 is alleged.

Particulars: paragraphs 238, 295, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART D - THE QUESTIONS FOR DETERMINATION

1. Did the Appellant sustain a personal injury of a psychological nature?
2. Did that injury arise out of, or in the course of, his employment, and was his employment a significant contributing factor to it?
3. Which of the circumstances pleaded in Part B.2 are management action taken by or on behalf of the employer in connection with the Appellant's employment, and which are conditions in which the work had to be done?
4. In respect of those circumstances that are management action, was that action reasonable and was it taken in a reasonable way?
5. Does section 32(5)(a) of the Act exclude the Appellant's psychological disorder from the definition of injury, having regard to the circumstances found not to be management action, and to any management action found not to be reasonable or not to have been taken in a reasonable way?

PART E - ORDERS SOUGHT

1. That the appeal be allowed.
2. That the decision of the Respondent dated 24 October 2024, Review Decision 69983, be set aside.
3. That it be declared that the Appellant sustained an injury within the meaning of section 32 of the *Workers' Compensation and Rehabilitation Act 2003*, being a psychological injury arising out of, or in the course of, his employment with Metro South Hospital and Health Service, employment being a significant contributing factor to that injury.

4. That the Appellant's application for compensation be accepted, and that the matter be remitted to the Respondent for assessment of the Appellant's entitlement to compensation consequent upon that acceptance.

5. Such further or other order as the Commission considers appropriate.

6. That the Respondent pay the Appellant's costs of the appeal under section 558(3) of the Act and section 191 of the Workers' Compensation and Rehabilitation Regulation 2025, including an award under section 191(3) of up to 1.5 times the amounts under section 191(2)(a), to be assessed failing agreement; alternatively, that costs be reserved.

PART F - COSTS

1. Costs of the hearing are in the Commission's discretion: section 558(3) of the Act; section 191(1) of the Workers' Compensation and Rehabilitation Regulation 2025. Costs ordinarily follow the event.

2. If the appeal is allowed, the Appellant will seek an order that the Respondent pay his costs of the appeal, assessed under section 191(2): (a) counsel's or solicitor's fees, for any period in which the Appellant is legally represented, at the costs stated in the Uniform Civil Procedure Rules 1999, schedule 2, part 2, scale C; (b) costs in relation to witnesses at the allowances stated in the Uniform Civil Procedure (Fees) Regulation 2019, part 3, including the fees and expenses of the medical practitioners called under paragraph B.1.11; and (c) any costs in relation to bailiffs.

3. The Appellant will contend that the amounts under section 191(2)(a) are inadequate and will seek, for any period in which the Appellant is legally represented, an award of up to 1.5 times those amounts under section 191(3), in total or for any item, having regard to: (a) the work involved: a record of 303 facts of which 298 are admitted, the documents at Annexure A to the notice to admit, non-party disclosure obtained under rule 64G, two notices to admit facts and a notice to admit documents, and the outlines of four witnesses served by the Respondent; and (b) the importance, difficulty and complexity of the matter: an appeal about a psychological injury under section 32(5)(a), which requires each pleaded circumstance to be classified, a finding on the reasonableness of each that is management action, and the weighing of the whole.

4. On the exercise of the discretion under section 558(3), the Appellant will also rely on the course of the proceeding, including that paragraph 27 of the Respondent's amended statement of facts and contentions identifies no management action [¶ 303], that the Respondent has notified no medical or expert evidence (Part B.6.5), and that the Respondent's own review decision found the rostering of 17 and 18 March 2024 to be unreasonable management action [¶ 260].

5. If the fees and expenses payable to a medical practitioner, or to any witness who gives evidence of a professional nature, are more than the costs awarded for that witness, the Appellant will apply to the Respondent under section 192 of the Regulation for payment of an additional amount.

6. The Appellant reserves the right to rely, on the question of costs, on any offer made without prejudice save as to costs.

Dated: _____ 2026

Cory Lea Shepherd

Appellant, self-represented